Traumatic Arrest

TB-10 | Trauma & Burns

Quick flow

01 Arrest With Trauma [EMT / AEMT / Paramedic]

Confirm apnea, pulselessness, rhythm when appropriate, DNR/MOST, and obvious-death criteria. If trauma does not clearly explain the arrest, use the age-appropriate medical cardiac-arrest pathway: adult cardiac arrest or PC-04 Pediatric Pulseless Arrest, while evaluating reversible causes.

02 Withhold Resuscitation? [EMT / AEMT / Paramedic]

Withhold only for a valid DNR/MOST or unmistakable death: decomposition, rigor mortis, dependent lividity, decapitation, hemicorporectomy, or injury incompatible with life. Do not use downtime alone as a withholding rule.

03 Correct Reversible Causes [EMT / AEMT / Paramedic]

Catastrophic hemorrhage • oxygenation/ventilation • Paramedic chest decompression when indicated • pelvic stabilization only when indicated • hypothermia prevention.

04 CPR / Defibrillation [EMT / AEMT / Paramedic]

Perform high-quality CPR and defibrillation for a shockable rhythm without delaying catastrophic hemorrhage control, effective oxygenation/ventilation, or indicated Paramedic chest decompression.

05 ROSC or Transport Candidate? [EMT / AEMT / Paramedic]

Witnessed arrest, organized rhythm, signs of life, feasible definitive-care transport, or an immediately reversible cause.

06 Appropriate Destination [EMT / AEMT / Paramedic]

Follow Medical Control and deceased-person policy. Apply the trauma field-triage and destination plan: high-risk patients to the highest-level appropriate trauma center within regional constraints; favor pediatric-capable trauma care for children. Use a closer stabilization hospital only for an immediate critical need that cannot safely be managed during longer transport, under the destination plan/Medical Control. Notify early, document the reason, and activate the predefined transfer process. Do not delay ground movement solely while awaiting aircraft.

Overview

- Apply the approved traumatic-arrest withholding and resuscitation criteria while prioritizing immediately reversible traumatic causes.
- When resuscitation is indicated, perform essential interventions without delaying movement toward definitive care for nonessential procedures.

Provider-Level Actions

Perform all immediately indicated care within scope without delaying rapid trauma transport.

EMT

- Confirm arrest and withholding criteria, control catastrophic hemorrhage, support airway/ventilation, apply AED/monitor, stabilize the pelvis only when indicated, and prevent hypothermia.
- Begin rapid transport when resuscitation criteria are met; do not delay for procedures that can occur en route.

AEMT

- Perform all EMT care plus IV/IO access during transport. Do not perform chest decompression; assist the Paramedic with the procedure when requested.

- Use authorized airway adjuncts and reassess for signs of life and ROSC.

Paramedic

- Perform all prior care plus advanced airway management, rhythm treatment, and chest decompression when tension physiology is suspected within standing orders. Consider bilateral decompression when the mechanism, examination, or physiology supports it.
- For ADULT clearly traumatic arrest, do not routinely give epinephrine. If used under local orders, it must not precede indicated lifesaving trauma interventions. Pediatric medication and resuscitation rules require the separate age-appropriate director-approved pathway; do not extrapolate the adult epinephrine instruction to children.
- Continue resuscitation or terminate only under current Claiborne policy and Medical Control requirements; do not use an app-only downtime cutoff.

Indications

- Patient with suspected traumatic arrest requiring prehospital assessment or treatment.

Warnings / precautions

- Do not apply traumatic-arrest withholding criteria to lightning strike, drowning, or hypothermic arrest; initiate resuscitation.
- Do not hyperventilate. Avoid delaying transport for endotracheal intubation or vascular access.
- Chest decompression is Paramedic-only under the current Tennessee statewide protocol guidelines. Do not extend this procedure to an AEMT without formal state authorization.

Clinical pearls

- Treat the trauma triad of death by controlling hemorrhage, limiting scene delay, and aggressively preventing hypothermia.
- Withholding is limited to a valid directive or unmistakable death; an initiated resuscitation requires Claiborne policy and Medical Control for termination decisions.

Related pathways / contacts

AC-03 Adult Cardiac Arrest - Adult medical-arrest pathway when trauma does not clearly explain arrest.

PC-04 Pediatric Pulseless Arrest - Age-appropriate pediatric medical-arrest pathway when trauma does not clearly explain arrest.

TB-06 Multiple Trauma - Hemorrhage control, provider-specific chest care, and destination priorities.

Source and references

Tennessee Emergency Medical Services ALS/BLS Blended Protocol Guidelines 2024-2025 - Adult Traumatic Cardiac Arrest and Chest Decompression procedure. | NAEMSP, ACS Committee on Trauma, and ACEP. Prehospital Management of Adults with Traumatic Out-of-Hospital Circulatory Arrest, March 2025. | Claiborne County EMS medical-director approval, October 17, 2026.

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